VICTORYAI

AI-Powered OUD Recovery *Companion & Outcomes Analytics Platform*

BayMark Health Services — OUD Pilot Opportunity

Prepared April 2026 · www.getvictoryai.com

CONFIDENTIAL · v2 — REVISED & SOURCE-CITED

THE OPIOID CRISIS

BayMark treats the hardest cases. *VictoryAI makes outcomes visible.*

PAYER PRESSURE

Outcomes are now required

DHCS and DMC-ODS require measurable MAT adherence and retention data. Programs without data-backed results face contract risk.

<20% of US adults with OUD receive any evidence-based MAT.

(SAMHSA NSDUH 2023)

RETENTION GAP

MAT attrition is the #1 leak

Opioid Treatment Program retention is the dominant business and clinical problem in MAT — published baselines hover near coin-flip levels.

40–60% of MAT patients drop out within 6 months.

(SAMHSA TIP 63)

MORTALITY STAKES

Every dropout is a mortality risk

OUD has the highest outpatient mortality rate in medicine. The cost of attrition is measured in lives, not just lost program revenue.

~107,000 US overdose deaths annually.

(CDC NCHS 2024 provisional)

COST OF INACTION

The real leak isn't billing codes. *It's retention.*

40–60%

MAT 6-month attrition (SAMHSA TIP 63)

$4–8K

Per-patient program revenue lost when a patient drops out

0

Between-visit risk visibility for most MAT clinics today

<20%

Of US adults with OUD receive evidence-based MAT

What this means for BayMark: A 10% improvement in 6-month MAT retention across BayMark's California footprint preserves materially more program revenue than any digital billing-code uplift. The retention story is the business case. Code optimization is upside, not the headline. *Sources: SAMHSA TIP 63; CDC NCHS overdose mortality 2024 provisional.*

THE SOLUTION

Three integrated layers, one *outcomes engine.*

PATIENT EXPERIENCE

AI Companion

- Sage AI companion (24/7, OUD-trained)
- MAT tracker (Suboxone / Methadone / Naltrexone)
- COWS withdrawal monitoring
- Daily check-ins: mood, cravings, pain
- 50-lesson CBT workbook
- Family Reward Board & gamification
- Drug screen + group session check-in
- SBIRT screening

CLINICAL WORKFLOW

Provider Dashboard

- Cohort-level engagement & risk view
- Early-warning signals from engagement decline
- MAT adherence trend monitoring
- Pain-craving correlation analytics
- Care plan documentation support (CCM/PCM)
- Drug screen & group attendance tracking
- Session prep summaries
- Adverse event escalation protocol

REVENUE VISIBILITY

Code Eligibility Tracking

- RTM (98980/98981) prerequisite tracking
- G2213/G2214 OUD treatment continuation
- CCM/PCM time documentation support
- TCM post-discharge tracking (99495/96)
- Group therapy attendance (H0005/90853)
- UDS documentation (G0480 family)
- Kipu data export — clinic team submits claims
- No billing through platform

REVENUE INTELLIGENCE

Capture eligible data. *Your team bills.*

VictoryAI captures structured patient engagement data and exports it to your EMR. Your billing team reviews, validates, and submits claims. We are not a billing vendor and we do not guarantee reimbursement.

WHAT WE TRACK

Structured engagement data

- Daily MAT adherence and dose confirmations
- COWS scores and craving/pain logs
- CBT workbook session completion + duration
- Care management interaction time
- TCM post-discharge contacts
- Group session attendance and UDS documentation
- Family engagement / reward redemption events

HOW IT FLOWS

Kipu integration

- Real-time data sync to Kipu (Enterprise tier)
- Code-prerequisite flags surfaced for billing review
- Audit trail of timestamps and clinician interactions
- Data segregation honoring 42 CFR Part 2
- HIPAA-compliant transport (BAA in place)
- Provider billing team submits all claims
- Effective realized estimate: $80–$200/pt/mo

ENGAGEMENT ENGINE

The Family Reward Board.
Designed for retention.

Family members pledge real rewards — gift cards, dinners, experiences — tied to recovery milestones. Patients earn points through MAT logging, check-ins, CBT sessions, and pain management. The mechanism (contingency management + social support) is well-documented in the addiction literature; the per-app effect size will be measured in the BayMark pilot.

1

Invite

Patient invites family / sponsor

2

Pledge

Supporters pledge rewards

3

Earn

Patient earns points via engagement

4

Unlock

Reward redeemed at threshold

5

Notify

Both sides celebrate together

The evidence we are NOT making up: Contingency management has Cochrane-level evidence in stimulant and opioid use disorders. Family-based interventions improve treatment retention in adolescent and adult SUD populations. We do *not* yet have peer-reviewed retention effect sizes for the VictoryAI implementation specifically — the BayMark pilot is designed to generate them under IRB oversight.

BUILT FOR OPIOID USE DISORDER

Not general addiction software. *Purpose-built for OUD.*

MAT + Drug Screen Tracking

Suboxone, Methadone, and Naltrexone adherence with dose confirmations. UDS results logging supports G0480-family toxicology documentation handled by your billing team.

COWS + TCM Transitions

Clinical Opiate Withdrawal Scale monitoring with provider sync. Post-discharge transition tracking captures the documentation prerequisites for 99495/99496 TCM (~$201–273 per qualifying event when prerequisites are met).

CCM/PCM Care Plans + AI

Auto-tracks between-visit engagement time to support CCM (99490, ~$62/mo) and PCM (99424, ~$82/mo) when ≥20 min/month is documented and time is not double-counted with RTM.

Group Therapy + Family Rewards

Group session check-in supports H0005/90853 attendance documentation (~$27–45/session). Family Reward Board drives engagement; effect on retention will be measured in the BayMark pilot.

HOW IT WORKS

Five steps. *From data to documentation.*

1

Patient logs daily data

MAT doses, cravings, pain, mood, COWS, CBT

2

AI structures insights

Risk patterns, sentiment, code prerequisite flags

3

Clinician reviews

Dashboard alerts, session prep, escalation protocol

4

Documentation generated

Code-eligible data, audit trail, time logs

5

EMR receives data

Auto-sync to Kipu. Billing team submits claims.

All claims are submitted by your billing team via Kipu. VictoryAI surfaces the documentation — your team determines medical necessity and bills.

OUD REVENUE INTELLIGENCE

Effective realized estimate *per OUD patient, per month.*

Below: defensible per-code ranges from a payer-blended (70% Medi-Cal / 30% commercial) model with ~65–70% engagement realization. These codes do NOT all bill concurrently — see notes.

Code	Description	How VictoryAI helps	Est. $/Pt/Mo
E/M Uplift 99213–99214	Better-documented MAT visit capture	Engagement data supports higher-complexity visit documentation	**$15–35**
RTM 98980 / 98981	Remote Therapeutic Monitoring (primary driver)	Engagement + ≥16 days data + interactive comm. (QHCP order required)	**$45–90**
G2213 / G2214	OUD treatment initiation / continuation	MAT engagement + ongoing adherence documentation	**$30–50**
CCM / PCM 99490 / 99424	Care management — when ≥20 min/month documented	Coordination time logs (cannot double-count with RTM)	**$20–50**
CHW / SDOH G0019 family	Community Health Worker — Medi-Cal only	Navigation, adherence, recovery coaching documentation	**$10–30**
EFFECTIVE REALIZED ESTIMATE			**$80–200 / patient / month**

Important: RTM, CCM, and PCM cannot double-count time. CoCM (99492/93) and BHI (99484) are mutually exclusive and require a defined care model with a psychiatric consultant — excluded from this model. Medi-Cal coverage is fragmented (DMC-ODS carve-outs apply). All claims subject to payer contracts, prior authorization, medical necessity, and your billing team's documentation. See deck disclaimer page.

FINANCIAL IMPACT — DEFENSIBLE MODEL

What the math actually says *for BayMark CA.*

$118
Effective realized PMPM
(blended payer mix)

1.6×
Net ROI on platform cost
(base case scenario)

Pilot
Retention lift to be measured
under IRB in BayMark pilot

Scenario	Realized PMPM	Annual / 100 pts	Annual / 500 pts	Net ROI*
Worst case (low engagement, lower reimbursement)	**$60**	$72,000	$360,000	**0.3×**
Base case (most likely — ~68% engagement)	**$118**	$141,600	$708,000	**1.6×**
Best case (>80% engagement, optimized RTM workflows)	**$200**	$240,000	$1,200,000	**2.0–2.5×**

**Net ROI assumes blended platform cost ~$45 PMPM. All figures are estimates, not guarantees. Claims depend on payer contracts, eligibility, prior authorization, and your billing team's documentation. Retention preservation value is incremental and not included in the table above — it is the larger of the two effects.*

POST-PILOT PRICING

Transparent pricing. *Defensible ROI.*

PATIENT APP — FREEMIUM

Premium

$19.99 /user/month

MAT tracker, COWS monitoring, Sage AI, CBT workbook, family rewards, gamification, Narcan hub, voice journal. Free tier remains free forever for crisis tools and basic recovery tracking.

ENTERPRISE PLATFORM

Custom

Quoted per facility

Provider dashboard, Kipu integration, white-label app, code-eligibility tracking, dedicated implementation, clinical safety governance support, and 42 CFR Part 2 compliance work as part of the implementation engagement.

CLINICAL LEADERSHIP & COMPLIANCE

Clinician-led. *Compliance-first.*

CLINICAL DIRECTOR & STUDY LEAD

[Name pending advisor agreement]

Fractional Chief Clinical Officer

Required credentials before pilot signing:

- Board-certified Addiction Medicine (ABAM/ABPM-Add)
- Published OUD / addiction treatment researcher
- IRB-pathway experience (Advarra or WCG)
- ASAM-aligned protocol authorship
- Adverse event escalation protocol owner

HIPAA

AES-256 encryption, BAA framework, role-based access for provider-facing views.

42 CFR PART 2

Implementation engagement includes consent flow, data segregation, and breach protocol.

DMC-ODS / ASAM ALIGNED

Documentation aligned to California outpatient OUD standards.

KIPU EHR INTEGRATION

Native data sync with role-based access. Billing flows through your EMR — not us.

THE PILOT PROPOSAL

240 days. 3–5 California sites. *Co-authored outcomes data.*

WHAT'S INCLUDED

- Full VictoryAI OUD platform deployment (iOS + Android)
- Provider dashboard for each pilot clinic
- Sage AI companion (24/7, OUD-trained)
- Kipu integration (sandbox → production cutover)
- Commercial IRB submission (Advarra or WCG)
- 42 CFR Part 2 compliance work + attorney memo
- Clinical safety governance + adverse event protocol
- Fractional clinical lead through outcomes report
- Co-authored outcomes report (publication-ready)

PILOT STRUCTURE

Clinics	3–5 BayMark CA sites
Duration	240 days (deliverable-based)
Cohort	~200 OUD patients
Programs	MAT · IOP · OTP
Investment	$500,000 (capped credit clause)
IRB	Advarra / WCG commercial
Success	Pre-agreed retention, engagement, safety metrics
Outcome	Co-authored study + enterprise LOI option

WHY BAYMARK

Lighthouse customer. *Co-authored evidence.*

WHY WE CHOSE BAYMARK

- Largest MAT provider in the US & Canada — 300+ clinics
- Deep California footprint with multi-level OUD programs
- Existing DHCS reporting workflow we can supplement
- Kipu standardization enables clean integration path
- PE-backed: EBITDA-focused decision model favors ROI clarity
- Operational leadership that understands frontline MAT realities

WHAT BAYMARK GETS

- Lighthouse customer status — first published OUD outcomes
- Co-authorship on ASAM / AAAP conference submission
- Capped enterprise credit (50% / $250K) on Year 1 license
- Direct input on OUD product roadmap
- Strengthened DHCS reporting with structured engagement data
- Defensible PE-board narrative on tech-enabled MAT

STAKEHOLDER VALUE

Built for every leader *at BayMark.*

FOR THE PRESIDENT / CSO

Defensive positioning

- Lighthouse case study before competitors
- Co-authored OUD outcomes for ASAM/AAAP
- Differentiated story for DHCS conversations
- Defensive moat against tech-enabled upstarts
- PE-board narrative on margin expansion

FOR THE CFO

Realistic ROI

- $80–200/pt/mo realized billable estimate
- Base case net ROI ~1.6× on platform cost
- Capped enterprise credit (50% / $250K)
- Retention preservation = upside, not in model
- Audit-ready documentation reduces clawback risk

FOR CLINICAL DIRECTORS

Real workflow fit

- Between-visit engagement visibility
- Early-warning signals from engagement decline
- MAT adherence + COWS trend monitoring
- Adverse event escalation protocol included
- Family-anchored retention mechanism

Investment: $500K — *capped credit, not free build.*

FUND THE PILOT

$500,000 — 240-day deliverable-based pilot

- Full platform build, IRB, 42 CFR Part 2 work
- 3–5 CA sites · ~200 patient cohort
- Fractional clinical lead through outcomes report
- Adverse event escalation protocol
- Co-authored ASAM / AAAP submission

- Milestone payments tied to deliverables, not dates
- M1 $150K signing · M2 $150K integration · M3 $100K enrollment · M4 $100K outcomes
- Capped credit: up to 50% / $250K toward Year 1 license
- 180-day window post-outcomes report
- Joint IP / data ownership terms

THE QUESTION EVERY BUYER ASKS

“Why not have Kipu *build this?*”

This is the most common objection — and the right one to ask. Here's the honest answer.

WHAT KIPU IS

System of Record

EMR core for SUD treatment programs

- Documentation, scheduling, billing
- Census and revenue cycle management
- Compliance reporting infrastructure

Roadmap focus: EMR features, not patient-facing engagement or AI predictive analytics.

WHAT VICTORYAI IS

System of Engagement

Patient-facing engagement layer

- Daily MAT logging and check-ins
- Family-anchored retention mechanism
- Engagement decline early warnings

Built specifically for OUD with COWS, MAT adherence, and clinical safety governance — not a general wellness app.

TOGETHER

Integration, not competition

VictoryAI integrates INTO Kipu — we are not a substitute for it

- Engagement data flows to Kipu in real time
- Code prerequisite flags surface in your existing billing workflow
- Your team submits all claims through Kipu
- [Kipu integration partnership: pending LOI]

THE TEAM

Small team. *Honest about the gaps.*

We are pre-revenue, pre-MVP, and a small team. That is a feature, not a bug, for a co-development pilot — but we want BayMark to know exactly who they would be working with.

FOUNDER & CEO

Alpa Patel

Product, Strategy, Sales

Founder of VictoryAI / SkyAI Technologies. Leads product vision, BayMark relationship, and pilot execution. Direct contact: 424.410.3603.

CLINICAL LEAD

[TBD]

Fractional Chief Clinical Officer

Search active. Required: Board-certified Addiction Medicine (ABAM/ABPM-Add), published OUD researcher, IRB pathway experience. Named before pilot signing.

ENGINEERING LEAD

[TBD]

CTO / Head of Engineering

Search active. Required: healthcare integrations experience (Kipu, Epic, or similar), HIPAA / 42 CFR Part 2 architecture, mobile + backend full-stack.

COMPLIANCE COUNSEL

[Engaged]

External healthcare attorney

Retained for 42 CFR Part 2 architecture, HIPAA BAA framework, contract structure, and IP / data ownership terms. Name available under NDA.

RUNWAY & RISK REVERSAL

What happens to BayMark *if we don't make it.*

Pre-revenue startups carry counterparty risk. We are honest about it and we structure the deal to neutralize it.

PROTECTION 1

Deliverable-Based Milestones

BayMark pays only for completed work. Four payments tied to deliverables (signing, integration verified, first patient enrolled, outcomes report) — never to calendar dates.

PROTECTION 2

Source Code & Data Escrow

Source code and patient data export protocols held in escrow with Iron Mountain or NCC Group. If VictoryAI ceases operations, BayMark gets immediate access to migrate all patient data.

PROTECTION 3

Insurance Stack

Professional liability ($2M+), cyber liability ($5M+), errors and omissions, technology E&O. Certificates provided to procurement at contract execution.

PROTECTION 4

Cap Table & Runway Disclosure

Current runway, investor stack, and cap table available under NDA. The BayMark pilot meaningfully extends our operating window — funding is not a near-term existential risk.

PARALLEL PIPELINE

BayMark first. *Not BayMark only.*

We are running this same play against four other national OUD providers in parallel. This is real scarcity, not manufactured urgency — engineering capacity allows one strategic deployment in Q2 2026.

Target	Footprint	Owner	Stage
BayMark Health Services	300+ clinics, US & Canada	Webster Equity / BPEA (PE)	**Lead target — this deck**
Acadia Healthcare	200+ behavioral health facilities	Public (NASDAQ: ACHC)	Discovery scheduled
Behavioral Health Group (BHG)	100+ OTPs across 24 states	Vistria Group (PE)	Outreach in progress
Pinnacle Treatment Centers	130+ programs, 8 states	PE-backed, strong CA presence	Outreach in progress
CleanSlate Centers	70+ outpatient MAT centers	PE-backed	Outreach in progress

Why this matters to BayMark: Whichever provider signs first becomes the named lighthouse customer in the published outcomes data, the ASAM/AAAP submission, and the Series A investor materials. The other four read about it.

THE ASK

See the prototype.
Sign the LOI.

1. View the VictoryAI OUD prototype — patient app + provider dashboard.
2. Walk through the defensible ROI model with your billing director and CFO.
3. Validate pilot scope, cohort size, and clinical safety governance with your CMO.
4. Sign Letter of Intent to participate in the 240-day co-authored pilot.

VICTORYAI

Let's build the future of *OUD recovery — together.*

VictoryAI is a clinical technology platform — not a billing service, not an FDA-cleared device, and not a substitute for clinical judgment. Pre-revenue, pre-MVP. The BayMark pilot is our path to generating defensible outcomes data under IRB oversight.

NEXT STEPS 1. Prototype demo · 2. Pilot scope alignment · 3. LOI · 4. M1 signing & build

Alpa Patel · Founder & CEO · 424.410.3603 · alpa@skyaitechnologies.com

What this deck does — and does not — claim.

Defensible math methodology. All revenue figures in this deck are effective realized estimates derived from a 70% Medi-Cal / 30% commercial blended payer mix and a ~65–70% engagement realization rate. They are not theoretical maximum stacking. RTM (98980/98981) requires a QHCP order, qualifying device data, ≥16 days of monitored data, and at least one interactive communication per month. CCM (99490) and PCM (99424) cannot double-count time with RTM. CoCM (99492/93) and BHI (99484) are mutually exclusive and require a defined collaborative care model with a psychiatric consultant — excluded from this model. H2019 is state-Medicaid-specific and varies by plan. Medi-Cal coverage is fragmented across DMC-ODS and managed care; many digital health codes require prior authorization.

What VictoryAI is not. VictoryAI is not an FDA-approved or FDA-cleared medical device. It is not a billing service, not a healthcare provider, and not a substitute for clinical judgment. VictoryAI captures structured engagement data and exports it to your EMR — your billing team submits all claims and determines medical necessity.

Clinical claims. Family Reward Board / contingency management mechanisms have published evidence in addiction treatment generally. The VictoryAI implementation does not yet have peer-reviewed effect-size data; the BayMark pilot is designed to generate it under IRB oversight (Advarra or WCG). Any retention, engagement, or outcome figures cited as VictoryAI-specific in earlier marketing materials should be treated as design intent, not measured results.

Sources. SAMHSA Treatment Improvement Protocol 63 (Medications for Opioid Use Disorder); SAMHSA NSDUH 2023; CDC NCHS 2024 provisional overdose mortality; CMS CY2024 Physician Fee Schedule; CMS Care Management code documentation requirements.